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Intersecting Inequalities: The Role of Caste and Socioeconomic Status in Maternal Healthcare Access

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Intersecting Inequalities: The Role of Caste and Socioeconomic Status in Maternal Healthcare Access

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Abstract

The caste system in India has historically been associated with discrimination against and marginalisation of select groups of people, particularly those considered outside the Hindu varna system, named the Dalits, and the native tribes of India. Legislative developments in the form of reservations designed as affirmative action aimed to develop these disempowered groups to a social standing equal to those belonging to the forward castes. And yet the current situation seems to be a long way from that. This paper aims to examine the intersection of caste and economic status in determining maternal healthcare accessibility in India, and the association between the two parameters, if any. Through a mixed-methods approach, data was collected from 109 participants across urban and rural regions, from forward castes (FC), Scheduled Castes (SC), Scheduled Tribes (ST), and Other Backward Classes (OBC). Analysis revealed significant disparities in access to maternal healthcare based on caste and socioeconomic status. Women from SC, ST and OBC communities, predominantly residents of rural areas, reported inferior access to maternal healthcare compared to their FC counterparts who mostly reside in urban settings with better resources. Economic status emerged a critical determinant, with higher-income groups accessing private healthcare and comprehensive ante-natal and post-natal services, while lower-income groups were reliant on government healthcare facilities, and in instances, received inadequate ante-natal and post-natal care. The study suggests that despite constitutional safeguards, socioeconomic disparities associated with caste impact the quality of maternal care received during and after pregnancy.

Keywords: Caste system, economic status, maternal healthcare, socioeconomic

disparities, healthcare disparities

1. Introduction

Social stratification in India can be traced back to the extermination and enslavement of the native, pre-historic Indian races of the Dravidians, Goetz, Nehru, and Munshi by the migrating Aryans in 1400 BCE. The Black Dravidians were called the Dasa Varna (slave caste), as compared to their masters the Arya Varna (the master class). The Shudras or slaves were a mixed-race people resulting from their intermarriage. (Ebenezer Sunder Raj, 1985). The Hindu sacred texts segregate society into four Varnas representing parts of the Purusha: the Brahmins, teachers and priests representing the head; the Kshatriyas, warriors and rulers representing the shoulders; the Vaishyas, traders and craftsmen representing the thighs; and the Shudras, labourers and craftspeople representing the feet. The fifth and unofficial class, the avarnas, were the lowest stratum of the society, were historically considered polluting, impure, doomed, and rendered untouchables, more popularly known as Dalits (Thapa, 2021). The stamp of religion on this system of segregation and slavery perpetuates a vicious cycle where Dalits are limited to low-status occupations like scavengers, sweepers, rag pickers, and coolies, which further feeds into the religious stamp of impurity (Vina M. Goghari and Mavis Kusi, 2023).

Over two millennia, the caste system has adapted but retained basic features. Economic and political insecurities, such as during colonial times, have often intensified caste divisions (Thapa, 2021). Despite individual economic success, caste status largely determines social standing. In the present day, the Indian government classifies castes into four main categories: Scheduled Castes (SC): Often referred to as ex-untouchables, they occupy the lowest social status.

Scheduled Tribes (ST): Comprising about 400 tribal groups, also at the bottom of the hierarchy.

Other Backward Castes (OBC): Groups historically disadvantaged but not as severely as SC and ST.

Forward Castes (FC): Higher status groups.

2. Aims and objectives

This research aims to:

- Assess the disparity in maternal healthcare across various economic and caste groups, and,
- Examine the relationship between caste and socioeconomic standing.

3. Statement of purpose

Social injustices faced by low castes, including violence and discrimination, directly impact health outcomes (Thapa, 2021). Children from SC and ST backgrounds have higher mortality rates compared to those from higher castes (Dommaraju, Adadjanian and Yabiku, 2008). Mohindra et al, 2006 finds a higher prevalence of poor health amongst SC, ST and OBC women, with socioeconomic variables having a magnifying effect. This study aims to examine the intersection of caste and economic status in determining maternal healthcare accessibility in India, and the association between the two parameters, if any.

4. Literature review

Despite constitutional provisions aimed at uplifting SC and ST populations, such as Article 46, poverty rates remain disproportionately high. For instance, studies have shown that poverty is significantly higher among Scheduled populations compared to non-Scheduled groups, with SC households often earning substantially less than their Forward Caste counterparts. According to the Global Multidimensional Poverty Index (GMPI), 33.3% of people in SC households are multidimensionally poor, compared to 15.6% of people of other castes. A

study using the National Family Health Survey data also found that 37% of children in SC households are multidimensionally poor, compared to 22% of children in other households.

While some argue that improving the socioeconomic status of low castes will eliminate healthcare inequalities, this view overlooks the unique role of caste as a social hierarchy that operates independently of economic status. Numerous studies indicate that social status influences health outcomes. According to Rama Baru, Arnab Acharya, Sanghmitra Acharya, A K Shiva Kumar and K Nagaraj (2010), factors such as individuals' perceptions of their social standing and the concept of social capital can affect access to healthcare and overall health. Lower-caste individuals often face significant barriers in accessing health services due to discrimination and systemic inequalities. Disparities in healthcare access are exacerbated by the spatial distribution of health facilities. Low-caste populations often live in areas with fewer healthcare resources. When facilities are available, discrimination from healthcare providers can further limit their access. Low-caste individuals may have less social capital as well, which affects their ability to access information about healthcare innovations and preventive measures for child health. This is reflected in lower utilization of antenatal and delivery care services. They generally have lower educational attainment due to structural barriers and internalized low self-worth. High poverty rates and occupational segregation further entrench these disadvantages. Poor neighbourhoods often have limited resources and health infrastructure, amplifying caste-related disparities. In contrast, affluent communities may offer better healthcare access and knowledge transfer, potentially reducing caste-based differentials in maternal health care quality and accessibility.

According to National Institutes of Health (NIH), Scheduled Castes (SC) and Scheduled Tribes (ST) face the greatest disadvantages in receiving antenatal care compared to Forward Castes

(FC). The European Journal of Midwifery also indicates that SC/ST women have notably lower odds of receiving antenatal care than their FC counterparts, while Other Backward Castes (OBC) also show significant disparities.

Similar trends are observed in delivery assistance, with SC/ST women having much lower odds of receiving health assistance during childbirth compared to FC women. Factors such as residential segregation and the relative under-utilization of maternal healthcare services among low castes are possible explanations for these disparities. Additionally, cultural attitudes that regard pregnancy as a condition not requiring medical attention, as well as discrimination from healthcare providers, may further impede access. For instance, home visits by health workers were reportedly limited to forward caste households, suggesting potential biases in service delivery. The persistence of mortality differentials between Forward Castes (FC) and low castes (SC/ST and OBC) suggests a complex interplay of social dynamics that extend beyond mere economic status. Additionally, diverse cultural practices related to childbirth and health care, along with caste-specific differences in women's autonomy and gender bias, could further complicate the relationship between caste and child mortality.

5. Methodology

This part of the paper outlines the approach used during the research, including the process for data collection and the techniques applied for analysis.

1. Research method:

The research employs a quantitative approach, focusing on numerical data for analysis. A quantitative research approach is a method of collecting and analysing numerical data to understand, describe, and predict a phenomenon. Statistical analysis has been used to collect data in a manner that is objective, in order to obtain unbiased results.

2. *Research question:*

The aim of this study is to examine the relationship between the availability of medical assistance and the socioeconomic status (caste and class) of pregnant women. The key research questions addressed throughout the study are:

- What is the extent of inequality in maternal healthcare across different castes?
- What is the relationship between caste and economic condition, and does it influence the aforementioned inequality further?

3. *Participant inclusion criteria:*

Women who have given birth within the last five years from urban cities and rural villages were included in the research. Pregnant women admitted in the wards of government hospitals in Kolkata, and women residing in the slum named Harijan Palli in Kalyani were verbally interviewed in person as well. Google Forms were also circulated amongst university and school students, who provided information collected from the women in their families who happened to be our target demographic. Responses were received from residents across Dumdum, Haldia, Howrah, Jirat, Barrackpore, Kalyani, Jadavpur, Plassey and Purna Medinipur. A variety of places were chosen for the study to ensure that participants represent diverse geographical, social, economic, and cultural backgrounds, providing a broad range of data. A total of 109 participants were included in the research.

4. *Data collection design and analysis technique:*

For this study, a survey questionnaire was distributed across multiple schools and universities, and puerpera (women who have recently given birth) were asked the same questions in person to gather data on a larger scale. The survey consisted of 11 questions, and the responses were analysed manually to accurately document down the data, given the different

modes of information collection.

5. *Ethical considerations:*

This research adhered to ethical guidelines throughout the process. Participants' anonymity and confidentiality were maintained during both data collection and analysis. The data collected was used exclusively for research purposes. The study respected participants' views and opinions, ensuring that the process did not cause any offense. The research was designed ensuring that no one's sentiments are hurt. Special care was taken not to diminish anyone's personal identity.

6. **Data presentation:**

The results have been presented using various methods, including pie charts, tables, and bar graphs. The findings from the survey questionnaire are displayed underneath.

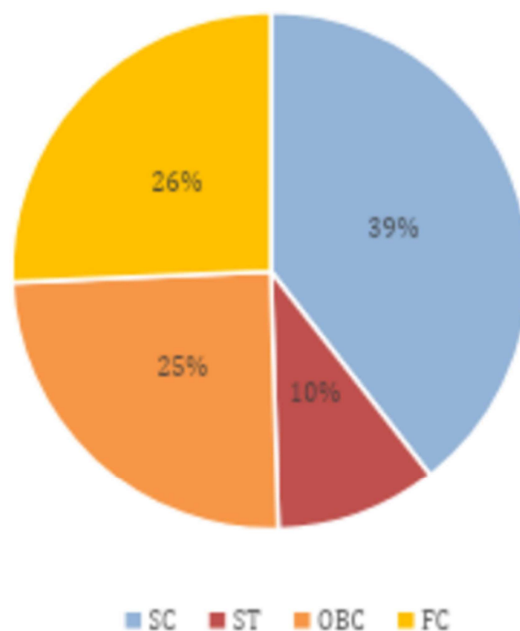


Figure 1: Graphical representation of the percentage of respondents from each category.

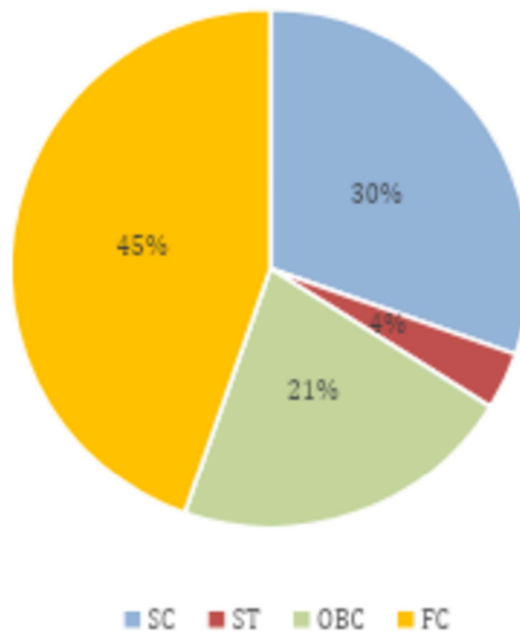


Figure 2.1 The percentage of people by caste who live in urban cities.

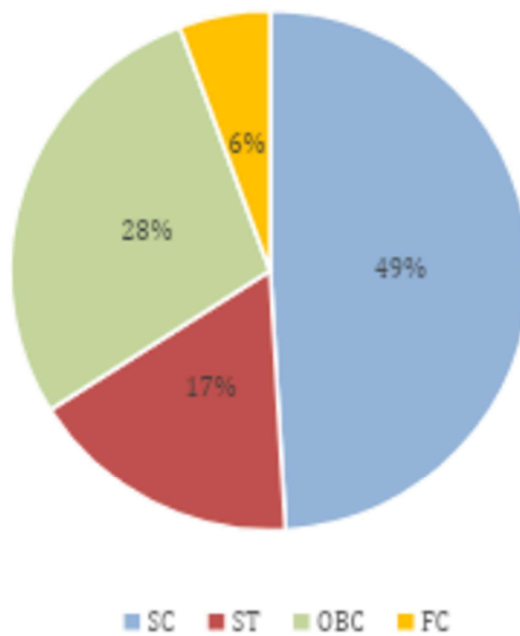


Figure 2.2 The percentage of people by caste who live in rural villages/underdeveloped slums.

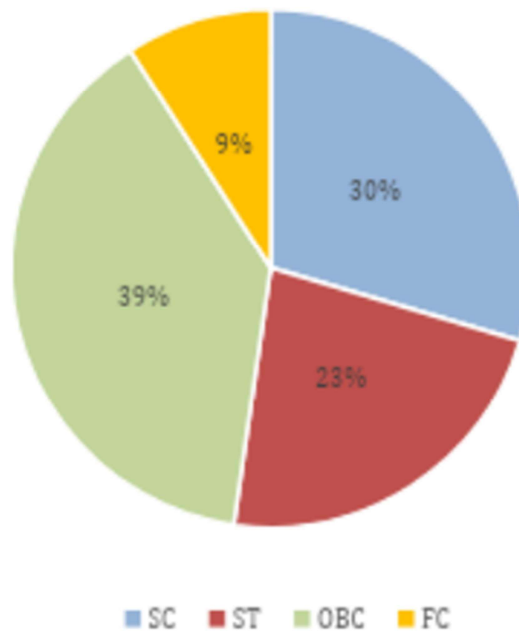


Figure 3.1 The percentage of participants by caste whose deliveries took place in government healthcare facilities.

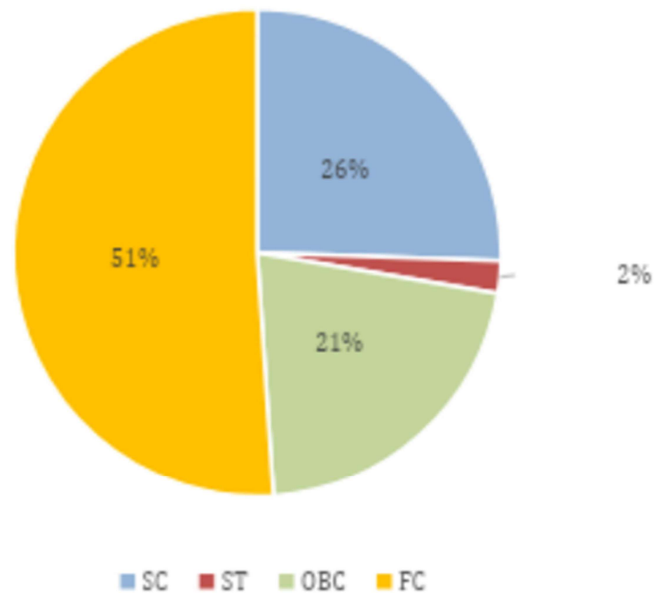


Figure 3.2 The percentage of participants by caste whose deliveries took place in private hospitals.

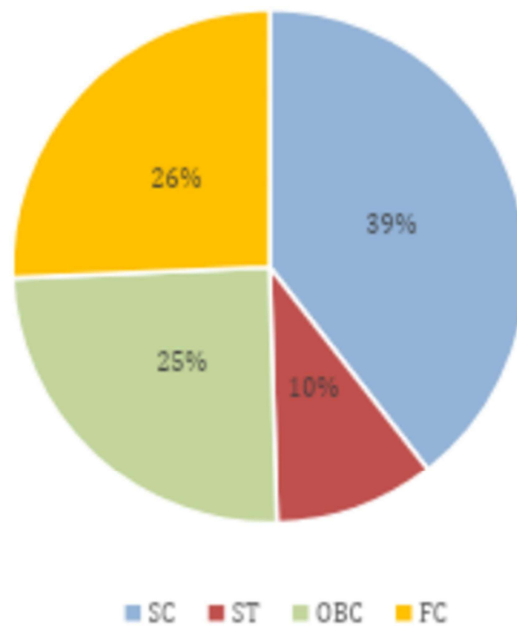


Figure 4. The percentage of births by caste that were attended to by medically trained personnel.

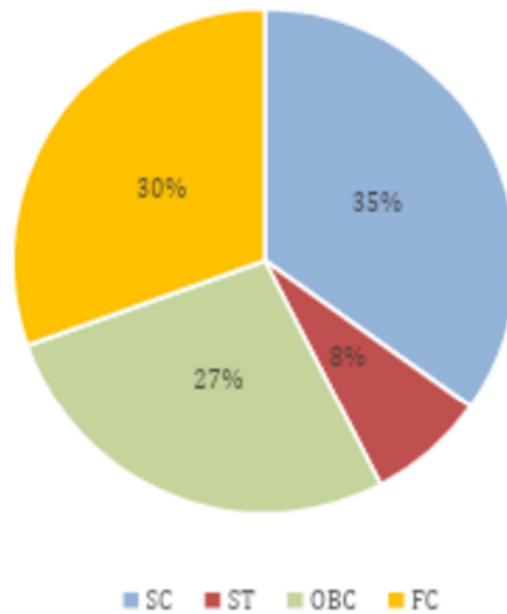


Figure 5. The percentage of women by caste who received at least 2 antenatal consultations from medically trained personnel.

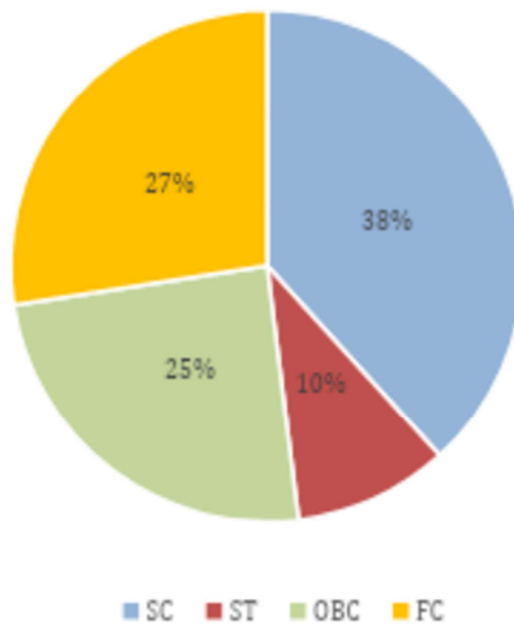


Figure 6. The percentage of women by caste who received at least one consultation from a

medically trained personnel in the period of puerperium (42 days after parturition).

Questions	Category			
	SC	ST	OBC	FC
How many people participated?	43	11	27	28
How many people reside in urban cities?	17	2	12	25
How many people reside in rural villages/underdeveloped slums?	26	9	15	3
How many deliveries took place in government healthcare facilities?	31	10	17	4
How many deliveries took place at a private hospital?	12	1	10	24
How many births were attended to by medically trained personnel (doctor, nurse or nurse-midwife)?	43	11	27	28
How many women received at least two antenatal care consultations (from the period of conception to delivery)?	32	7	25	28
How many women received at least one medical consultation in the period of puerperium (42 days after delivery)?	39	10	25	28

Table 1: Recorded responses of participants relating caste with place of residence and maternal

care indicators.

Questions	Annual income			
	<2.5	2.5-8	8-15	>15
	LPA	LPA	LPA	LPA
How many deliveries took place in government healthcare facilities?	19	33	7	0
How many deliveries took place at a private hospital?	0	1	39	10
How many births were attended to by medically trained personnel (doctor, nurse or nurse-midwife)?	19	34	46	10
How many women received at least two antenatal care consultations (from the period of conception to delivery)?	11	28	43	10
How many women received at least one medical consultation in the period of puerperium (42 days after delivery)?	15	32	45	10

Table 2. Relating maternal healthcare quality with economic status

Category	Annual Income
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	<2.5 LPA	2.5-8 LPA	8-15 LPA	>15 LPA
SC	11	16	13	3
ST	5	4	2	0
OBC	3	12	9	3
FC	0	2	22	4

Table 3. Recorded annual income of participants by caste

7. Results and discussion

The data reveals that accessibility to maternal healthcare varied significantly across caste groups. Among the 109 people surveyed, 56 of them reported being residents of urban cities while 53 belonged to villages and slums. Approximately 89.28% of people belonging to the Forward Castes in the survey resided in cities and townships, with a greater access to advanced healthcare facilities. On the contrary, over 60.46% people belonging to Scheduled Castes, 81.81% of those belonging to Scheduled Tribes and 55.56% of the ones from Other Backward Classes were the residents of villages, and have, in most cases, reported facing difficulties in accessing similar facilities as city dwellers.

While 85.71% of the pregnant mothers from forward castes gave birth in a private hospitals, ensuring better hygienic and comfortable conditions, 72.09% of the mothers from Scheduled Castes, 90.91% from Scheduled Tribes and 62.96% from Other Backward Classes had to opt for government hospitals to give birth. Furthermore, while 100% mothers from the FC group received at least two antenatal and one postpartum consultation, mothers from other categories did not. 25.58% mothers from the SC population participating in the survey did not receive the minimum of two medical consultations during their pregnancy, and neither did

36.37% women from the Scheduled Tribes and 7.4% from Other Backward Classes. As for a medical consultation during puerperium, 9.3% SC, 9.09% ST and 7.4% OBC mothers could not access it.

It was also observed that everyone belonging to the income group of >15 lakh per annum chose to give birth at a private hospital and received adequate healthcare both during and after the termination of the pregnancy/parturition, such was not the case for mothers from other economic groups. 100% mothers from the group that earned <2.5 lakh per annum gave birth at a government hospital, with 42.1% of them lacking antenatal care and 21% of them deprived of postpartum healthcare. For the income group earning between 2.5-8 lakh per annum, 97% gave birth in a government hospital, and 15.15% of them did not receive adequate antenatal care, while 5.88% lacked postpartum facilities as well. For families earning between 8-15 lakh per annum, healthcare conditions improved, though not at par with the highest income group. 84.78% of this group gave birth at a private hospital, with 93.47% and 97.82% receiving antenatal and postpartum healthcare, respectively.

A relationship could also be drawn between caste and economic status. It can be summarized as follows:

- Scheduled Caste: 25.58% of the participants earned <2.5 LPA, 37.2% earned between 2.5-8 LPA, 33.33% earned between 8-15 LPA, and only 7.69% earned >15 LPA.
- Scheduled Tribe: 45.45% earned <2.5 LPA, 36.36% earned between 2.5-8 LPA, 18.18% earned between 8-15 LPA, and absolutely 0% earned >15 LPA.
- Other Backward Classes: 11.11% earned <2.5 LPA, 44.44% earned between 2.5-8 LPA, 33.33% earned between 8-15 LPA, and 11.11% earned >15 LPA.
- Forward Caste: 0% earned <2.5 LPA, 7.14% earned between 2.5-8 LPA, 78.57% earned

between 8-15 LPA, and 14.28% earned >15 LPA.

8. Key findings

- Residents of cities mostly opted for private hospitals to give birth, while the majority of village and slum inhabitants chose government hospitals.
- Majority of the participants from the forward castes lived in cities, while the larger chunk of those belonging to the other caste groups hailed from villages or underdeveloped slums, pointing out quite starkly that the forward castes have better access to all sorts of facilities.
- Most women from the forward castes gave birth at private hospitals, with adequate access to antenatal and postpartum healthcare. Such was not the case for women from the other caste groups, which once again corroborates our hypothesis that the scheduled castes, scheduled tribes and other backward classes face marginalization and discrimination in terms of accessibility to healthcare in general, and maternal healthcare in particular.
- It was observed that with improving economic standing, the quality of healthcare facilities availed by pregnant women and new mothers improved simultaneously.
- It can be inferred that the economic conditions of the forward castes are markedly better than those belong to the other caste categories.

9. Conclusion

The study, involving 109 participants, found evidence supporting the hypothesis that there is a discrepancy between the accessibility to maternal healthcare facilities based on caste, substantiating that Scheduled Castes, Scheduled Tribes, and Other Backward Classes face remarkable prejudice and injustice regarding the same. The findings suggest that a larger proportion of participants from the forward castes resided in more developed areas, highlighting

the better access to resources enjoyed by them. In contrast, the marginalized classes could not benefit sufficiently from comprehensive antenatal and postpartum care. The study also found a correlation between improved economic status and better healthcare facilities for pregnant women and new mothers. The findings indicate that the economic status of the community is closely tied to caste composition. It could be further concluded that the economic conditions of the forward castes are significantly superior to those of other caste groups, which once again hampers their ability to access maternal healthcare. These results underline the necessity of addressing caste-based disparities in maternal health outcomes through targeted interventions, recognizing that socioeconomic improvements alone may not suffice. Improving healthcare availability and combating discrimination within healthcare systems are essential steps toward enhancing maternal and child health outcomes for marginalized caste groups. Understanding and mitigating the barriers faced by low caste women in accessing these services is vital as well.

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